INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Capecitabine + rectal radiotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
7 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00586
Version 3

> Take capecitabine twice daily on Days 1–5 of each 7-day cycle with radiotherapy, for a maximum of 5–6 cycles. Days 6–7 are not routine tablet days. Follow the team's written instructions if radiotherapy or tablets are interrupted.

Treatment stages

01 **Weekly chemoradiation**
Tablets + radiotherapy

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	5
03	Symptoms and supportive care	10
04	Sources and further information	17

My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Stop capecitabine and contact oncology for significant diarrhoea, painful mouth or hand-foot symptoms affecting activity, vomiting that prevents drinking, fever or sudden illness. Do not restart after a toxicity interruption until the team advises.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

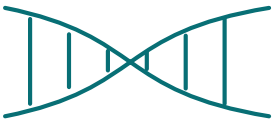
How the treatment works

Capecitabine tablets help pelvic radiotherapy work against locally advanced rectal cancer. Your team explains how this course fits with surgery and any other chemotherapy.

ORAL CHEMOTHERAPY

Capecitabine

Xeloda



Becomes fluorouracil in the body

After you swallow capecitabine, your body changes it into fluorouracil. This interferes with the deoxyribonucleic acid (the cell's genetic material) (DNA) and RNA processes cells use to grow. Normal cells can also be affected, which helps explain mouth soreness, bowel symptoms and changes in blood counts.

Swallowed tablet → Converted to fluorouracil → Cell growth disrupted

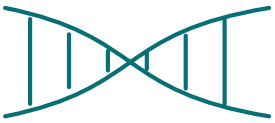
What to watch for

Stop tablets and contact the team for significant diarrhoea, painful mouth ulcers, hand-foot soreness affecting activities, fever or dehydration. Chest pain needs urgent assessment. Do not restart until instructed.

LOCAL RADIATION TREATMENT

Pelvic radiotherapy

External beam treatment



Targets the pelvic treatment area

Radiation is directed at the planned area. Capecitabine helps sensitise cancer cells to it. Nearby bowel, bladder and skin may also become irritated. Effects can build over the weeks and may continue after the final session.

Planned radiation field → Damage to cancer-cell DNA → Cells lose the ability to multiply

What to watch for

Tell the radiotherapy team about diarrhoea, pain, skin soreness or urinary changes. Discuss possible longer-term bowel, bladder, sexual and fertility effects before treatment.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim and duration

The same medicine can be used for different purposes.

Why this treatment?

Capecitabine tablets are given with pelvic radiotherapy for locally advanced rectal cancer. The tablets help the radiation treatment work; your team confirms how this fits with surgery and other treatment.

How long?

The NCCP weekly schedule runs for up to 5–6 cycles with radiotherapy. The radiation oncologist specifies the dose, field and visits; the multidisciplinary team (MDT) confirms later surgery or other therapy.

Alternatives and decisions

Discuss the expected benefit, burdens, reasonable alternatives and the option of supportive care. The best choice depends on your cancer, fitness and preferences.

Why this treatment is offered

Capecitabine was an effective oral alternative to infused fluorouracil when combined with radiotherapy in the study. Outcomes included the whole treatment programme, including surgery.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

How the treatment fits together

Capecitabine + rectal radiotherapy

7-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

TABLETS + RADIOTHERAPY

Weekly chemoradiation

D1 D5 D7

Capecitabine twice daily on Days 1–5 of each 7-day cycle, alongside radiotherapy, for up to 5–6 cycles. Follow the written plan for treatment days and interruptions.

The treating team confirms the aim, duration and next steps. The visual pattern does not replace your prescription.

BASELINE → TREATMENT → FOLLOW-UP

Treatment checks across the course

Relative stages, not equally spaced dates. Agree who books each check, reviews results and gives advice if a check is missed.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00586 v3 · official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Capecitabine + rectal radiotherapy: treatment and checks

Locally advanced rectal cancer with concurrent radiotherapy

Take capecitabine twice daily on Days 1–5 of each 7-day cycle with radiotherapy, for a maximum of 5–6 cycles. Days 6–7 are not routine tablet days. Follow the team's written instructions if radiotherapy or tablets are interrupted.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	During treatment	Phase / dose review	After treatment
Bloods and symptoms NCCP 00586	dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD), blood count, kidney/liver; agree radiotherapy (RT) and tablet plan	Confirm tablet days and meal instructions	Weekly/pre-cycle bloods; bowel, skin, urine and intake review	Do not restart or make up tablets without team advice	Radiation/surgical review and management of persistent effects
Treatment review NCCP 00586	Confirm exact cancer setting and biomarkers	Explain expected benefit and how it will be assessed	Report new symptoms between visits	Review response, tolerability and active medicines	Agree the next treatment or surveillance plan
Tablets and radiation NCCP 00586	Coordinate prescription, radiation bookings and transport	Twice-daily tablets on Days 1–5	No routine tablets on Days 6–7; maximum 5–6 cycles	Coordinate any tablet or radiation interruption	Completion does not mean all side effects end immediately

What each check is for

1 Confirm your personal plan

Before starting

Capecitabine tablets help pelvic radiotherapy work against locally advanced rectal cancer. Your team explains how this course fits with surgery and any other chemotherapy.

Ask / act: Agree medicines, treatment days, duration and monitoring.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Tablets, visits and practical planning

A simple guide to what happens before treatment, on treatment days and at home. Your own service confirms appointments and prescriptions.

STEP 1

Treatment phases and appointments

STEP 2

Checks after treatment

Keep reviewing bowel, bladder, skin, fatigue and nutrition effects during and after the course. Late effects and surgical assessment follow the radiation/colorectal team plan. Do not automatically extend tablets to make up missed radiation days or continue beyond the planned course.

STEP 3

Access and practical support

No intravenous (IV) port or infusion pump is needed for capecitabine itself; separate treatments may differ. Radiation appointments, tablet supply, meals and transport should be planned together.

Keeping tablets safe

Keep the original labelled packaging away from children. Follow the storage label and pharmacy handling instructions. Do not share tablets or discard them in household waste.

If treatment is interrupted

Tell the team which doses were missed and why. Never extend the course, double a dose or move tablet days yourself. Ask for a revised written plan.

Questions about your appointments

Ask about transport, blood-test location, pharmacy collection and a named contact for appointment changes. Booking arrangements differ between centres.

Radiotherapy planning and recovery

The radiation team plans the field and explains positioning and preparation. Treatment-related bowel, bladder and skin effects can continue after the last session; keep reporting them.

Tests and checks

Monitoring helps the team decide whether treatment can continue.

DPD testing

DPD helps break down fluoropyrimidine medicines. Low activity can cause severe toxicity; testing before treatment helps guide the plan but cannot prevent every serious reaction.

Blood, kidney and liver results

Counts, kidney and liver function are checked before each cycle. Diarrhoea and poor intake can worsen kidney function and make tablets harder to tolerate.

Your symptoms matter

Report bowel frequency, stoma output, mouth soreness, hand-foot pain, chest symptoms and what daily activities you can no longer manage.

Additional checks

Radiotherapy reviews also address pelvic skin, bowel, urinary and nutritional problems.

Review, recovery and follow-up

Your plan can change as treatment progresses.

Dose changes are individual

A pause or reduction may be needed because of symptoms, blood results or kidney function. The team considers the treatment aim and other options.

After the last dose

Keep reporting new or persistent symptoms after treatment ends. Your team confirms follow-up, scans and recovery support; the timeline is not a forecast.

Prepare for review

Bring questions, symptoms and missed-dose records. Tell the team what matters most to you and ask them to explain the next decision.

CHAPTER 03

Symptoms and supportive care

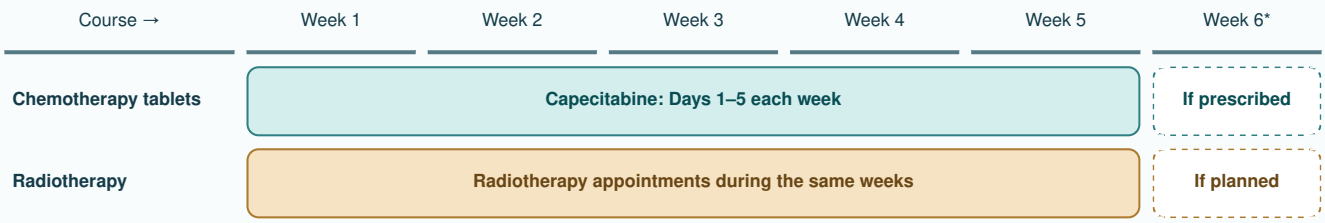
IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

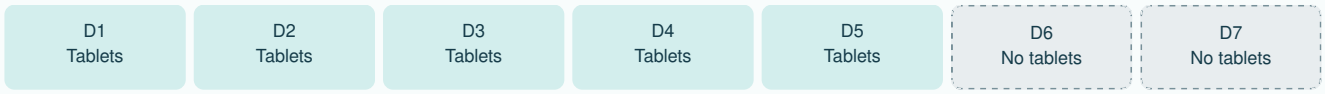
THE WHOLE TREATMENT COURSE

Chemotherapy and radiotherapy: given together

Capecitabine tablets and radiotherapy run alongside each other during this course. You do not need to enter any dates.



Inside each prescribed week



Capecitabine is taken twice daily on Days 1–5. The weekly tablet break is different from finishing the whole course. Radiation appointments follow the centre's plan. If a session changes or either treatment is paused, contact the team for the revised tablet instructions.

*The NCCP course is a maximum of 5–6 weekly cycles. The sixth week is shown as optional. Your radiation team confirms the actual number of sessions and any bank-holiday changes.

> **After the course: recovery and reassessment. Surgery or other chemotherapy, if planned, follows your individual multidisciplinary plan; no universal interval is implied.**

Educational reference, not a personal appointment calendar. NCCP 00586 v3, page 1

Day 1 → Day 7: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Capecitabine tablets	 **Days 1–5** Twice daily as prescribed
Radiotherapy visits	 **Days 1–5** Follow your appointment plan
Tablet-free days	 **Days 6–7** No planned capecitabine
Bowel / mouth / skin	 **Days 1–7** May develop or accumulate; report symptoms

Treatment or check	Timing in this view
Infection / feeling unwell	 **Days 1–7** Seek advice on any day
Pelvic skin / bladder	 **Days 1–7** Can build over the treatment weeks

Your written plan comes first: Capecitabine twice daily on Days 1–5 of each 7-day cycle, alongside radiotherapy, for up to 5–6 cycles. Follow the written plan for treatment days and interruptions.

Act when symptoms occur: Do not wait for the next visit or use the chart to decide a symptom is safe.

Symptoms can build: Bowel, mouth and hand-foot symptoms may accumulate and can continue during tablet-free days.

Across the course: Radiotherapy symptoms can build over successive weeks and continue after treatment finishes.

Why these bands stay broad: Treatment bands show the NCCP schedule. Symptom bands are broad teaching illustrations: they do not represent measured daily risks or predict when you will feel well. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00586 v3 · official regimen ↗ · eviQ: taking capecitabine and managing symptoms (NCCP defines the Irish schedule) ↗ · NCI: radiation side effects ↗

There are no guaranteed "safe days". Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Between-treatment notes

Day 1–5: Treatment days

Capecitabine twice daily on Days 1–5 of each 7-day cycle, alongside radiotherapy, for up to 5–6 cycles. Follow the written plan for treatment days and interruptions. Use the current tablet label. Swallow whole with water with or within 30 minutes of food; do not crush or cut tablets.

Day 6–7: Planned tablet-free days

Do not take leftover tablets to replace doses missed earlier. Side effects may continue during this interval. Keep the oncology contact plan available and confirm the next review before restarting.

Support around the medicines

1 Concurrent capecitabine and pelvic radiotherapy

No automatic high/moderate IV antiemetic course for capecitabine. Written tablet-day and missed/held-dose instructions; do not double doses or extend to weekends without the team's plan. Diarrhoea/dehydration, hand-foot and pelvic skin care; nutrition and urgent-contact support.

Taking capecitabine safely

Tablets are active chemotherapy, with a specific schedule.

Meals and doses

Use your current prescription. Swallow tablets whole with water with or within 30 minutes after food. Do not cut, chew or crush them; ask pharmacy if swallowing is difficult.

Missed doses or vomiting

Do not replace a missed or vomited dose. Never double the next dose. Contact the team for repeated vomiting or uncertainty about the schedule.

Pause and restart

Stop tablets for significant diarrhoea, painful mouth/hand-foot symptoms affecting activity, fever or dehydration and seek advice. A toxicity-related pause requires instructions before restart.

Different tablet strengths

Different strengths may be combined to make your dose. A reduction changes tablet numbers: use the new label and do not calculate a change from this page.

Your radiotherapy visits

The radiation team gives a separate, individual plan.

Before treatment

Planning appointments define the area to treat and how you will be positioned. Follow the exact bowel and bladder preparation instructions from your centre.

Daily sessions and tablets

NCCP 00586 specifies capecitabine Days 1–5 of each week. Confirm the plan if a session is missed, postponed or falls around a bank holiday; do not add weekend tablets yourself.

After the course

Side effects can accumulate and continue after treatment. Ask about follow-up and possible late bowel, bladder, sexual or fertility effects.

Recognise symptoms and act

Do not wait for the next appointment if you feel unwell.

Bowel and hydration

Report diarrhoea or increased stoma output early. Inability to drink, dizziness, very little urine, severe pain or blood in stool needs prompt assessment.

Hands, feet and mouth

Pain, cracking, peeling or ulcers that affect normal activities or eating/drinking need advice. Gentle preventive care helps, but does not replace reporting.

Chest symptoms and infection

Stop capecitabine and seek urgent assessment for chest pain. Fever, shaking chills or sudden illness needs immediate oncology contact, even if the temperature is normal.

Treatment-specific effects

Pelvic radiation can cause bowel, bladder and skin irritation, with possible later effects. Ask about sexual health and fertility before treatment.

Daily life and other medicines

Make the plan fit your practical needs.

Eating, drinking and activity

Aim for tolerable meals and fluids; contact the team if intake falls. Gentle activity and rest can be adjusted to your energy. Ask for dietetic help if needed.

Medicine and supplement checks

Check new medicines with oncology pharmacy. Warfarin can require closer international normalised ratio (blood-clotting test) (INR) checks; phenytoin levels may need monitoring. Brivudine is an important prohibited interaction.

Family, work and fertility

Ask about infection exposure, work, intimacy, contraception, fertility and any caring responsibilities. Use the medicine-specific duration of precautions given by your team.

Procedures and travel

Plan transport and support around appointments. Ask which symptoms or blood-count problems affect travel and when to contact the service.

Questions worth asking

What if I forget a dose or vomit?

Do not take an extra dose. Take the next planned dose at its usual time unless treatment has been stopped; report repeated vomiting and check with the team if unsure.

Why do I need blood tests if I take tablets at home?

Blood counts, kidney and liver results, together with symptoms, guide whether to continue and at what dose.

Do I need a port or a pump?

No take-home chemotherapy pump is used. Capecitabine is swallowed, so a central line is not required just to give it.

What happens after treatment?

Ask for your review and follow-up plan. Continue reporting symptoms after the last dose; further therapy depends on the treatment aim and response.

Try explaining the plan back: Which days do you take tablets? What do you do if you miss a dose? Which symptoms mean stop tablets and call? Who do you contact out of hours?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Concurrent capecitabine and pelvic radiotherapy

Active medicines: capecitabine + radiotherapy

Emetogenic risk: oral_minimal_low

Medicine	Dose / route	When
Rescue antiemetic	Individual prescription	As required for oral capecitabine Radiotherapy nausea risk depends on field/site and is reviewed separately by the radiation team.

Additional phase-specific support

- No automatic high/moderate intravenous (IV) antiemetic course for capecitabine.
- Written tablet-day and missed/held-dose instructions; do not double doses or extend to weekends without the team's plan.
- Diarrhoea/dehydration, hand-foot and pelvic skin care; nutrition and urgent-contact support.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Mouth

Capecitabine

Painful ulcers or reduced intake: stop tablets and call.

3 Bowel / stoma

Capecitabine + radiotherapy

Significant diarrhoea, increased output or dehydration needs prompt advice.

5 Blood / infection

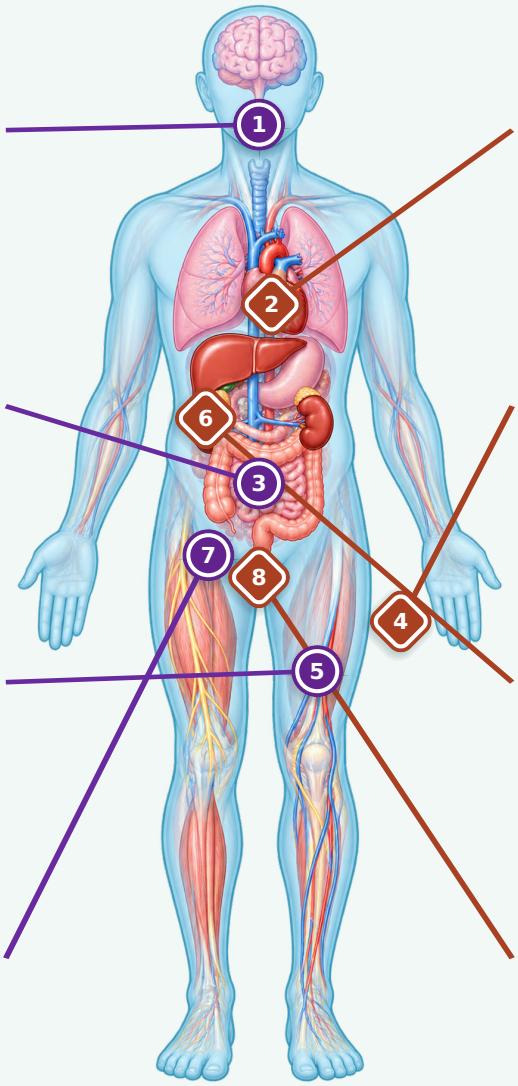
Capecitabine

Fever, shivers or sudden illness: contact oncology immediately.

7 Pelvic skin

Radiotherapy

Report painful, broken or weeping skin in the treatment area.



2 Chest

Capecitabine

New chest pain: stop tablets and seek urgent assessment.

4 Hands / feet

Capecitabine

Pain or peeling affecting daily activity: stop tablets and call.

6 Hydration / urine

Capecitabine safety

Reduced urine, dizziness or inability to drink needs assessment.

8 Bladder / sexual health

Radiotherapy

Report urinary pain and discuss fertility and sexual effects.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Stop capecitabine and contact oncology for significant diarrhoea, painful mouth or hand-foot symptoms affecting activity, vomiting that prevents drinking, fever or sudden illness. Do not restart after a toxicity interruption until the team advises.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00586 v3 · pNCCP00586 supportive care; classification V6 Table 5**
   https://healthservice.hse.ie/documents/6396/586_v3_Capecitabine_825_and_radiotherapy (RT).pdf
2. **eviQ: taking capecitabine and managing symptoms (NCCP defines the Irish schedule) ↗**
   https://www.eviq.org.au/medical-oncology/colorectal/rectal/84-rectal-locally-advanced-capecitabine-chemoradia/patient-information
3. **NCI: radiation side effects ↗**
   https://www.cancer.gov/about-cancer/treatment/types/radiation-therapy/side-effects
4. **Capecitabine product information: medicines, interactions and precautions**
   https://www.medicines.org.uk/emc/product/14545/smpc
5. **HSE: chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
6. **NCI: rectal cancer treatment**
   https://www.cancer.gov/types/colorectal/patient/rectal-treatment-pdq
7. **Hofheinz rectal chemoradiotherapy trial: primary trial report**
   https://pubmed.ncbi.nlm.nih.gov/22503032/
8. **NSABP R-04**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC4849360/
9. **NCCP emetogenic classification V6**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf
10. **NCCP NCIS Medical Oncology antiemetics V8, Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
11. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
    https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq